

Menopause and MS

What happens during menopause?

Most women go through menopause in their late 40s or early 50s, although a smaller number of women have an earlier menopause. MS doesn't seem to change the age when menopause happens.

During menopause, the levels of the reproductive hormones (estrogen and progesterone) gradually fall. A woman's menstrual periods eventually stop. After menopause, levels of natural estrogen and progesterone remain low.

Women can have symptoms in the years when their hormone levels are falling (perimenopausal) and afterwards (post-menopausal). Some women find that menopausal symptoms have a big impact on their life, while others don't.

The more common changes that occur during menopause are:

- Less regular periods
- Hot flashes, chills and night sweats
- A dry vagina (which can be uncomfortable and affect your sex life)
- Sleep problems
- Mood changes
- Memory problems
- Weight gain
- Thinner hair, drier skin
- Loss of breast fullness

How does menopause affect women long term?

After menopause, women commonly find that symptoms like hot flashes, mood changes and memory problems stop. However, some changes may continue, like changes in body shape, hair, and skin.

Post-menopausal symptoms

- Changes to sex drive
- Thinner hair, drier skin, and loss of breast fullness
- Weight gain

- Sleep problems
- Depression

Due to lower levels of estrogen and progesterone, after menopause women have a higher risk of other health conditions.

Increased post-menopausal health risks

- Cardiovascular disease
- Weaker, more brittle bones (osteoporosis)
- Reduced bladder control
- Bladder infections

Are your symptoms due to menopause or MS?

Some of the effects of menopause overlap with the symptoms of MS. For example, either can cause:

- difficulty sleeping
- mood changes, such as feeling low, anxious or irritable
- bladder problems
- memory problems, such as being forgetful or struggling to find the right word
- changes to sex life.

If you have symptoms that are causing you problems, speak to your primary care provider, neurologist or gynecologist (a specialist in the female reproductive system). Together you can work out the cause of your symptoms (whether it is menopause, MS or something else). This can make a difference to which treatments are most suitable for you.

What can help menopausal symptoms?

Although menopause is a natural part of life, you don't have to deal with its effects on your own. Many symptoms and risks can be reduced with treatments and self-help.

Treatments include:

- hormone replacement therapy (HRT)
- vaginal estrogen creams, skin patches, oral medications and rings (pessaries)
- medications to reduce hot flashes, improve bone health and improve sleep

These treatments have been tested in large groups of women but not specifically in women with MS.

HRT increases the levels of reproductive hormones in your body. It's usually very effective at reducing menopausal symptoms but has some risks. For women with troublesome symptoms who are younger than 60 years old, the benefits usually outweigh the risks.

Self-help techniques for menopause include:

- regular pelvic exercises to strengthen bladder control
- bedtime routines and techniques to improve sleep
- relaxation techniques to reduce stress and anxiety
- avoiding caffeine and alcohol to lessen hot flashes.

Talk to your primary care provider or gynecologist about the options that could help your symptoms.

Can menopause worsen MS symptoms?

Although there have been a few studies on this, the results don't give a clear answer.

For most women, menopause doesn't seem to affect MS greatly. For some women, hot flashes might temporarily worsen some MS symptoms.

Some women report their MS symptoms and level of disability got worse after menopause. As people with MS get older, they have fewer times when their symptoms come and go, and disability is also likely to increase.

It's not clear if women's MS symptoms get worse after menopause because they're getting older or because their hormones are changing.

How do menopause and MS affect bone health?

Women who have been through menopause are at risk of osteoporosis. This makes bones less dense, more fragile and more likely to break if you fall or have an accident.

MS can also increase the chance of osteoporosis. It's estimated that almost 2 in every 10 women with MS have osteoporosis. About 4 in every 10 show a smaller amount of bone weakness called osteopenia). Osteopenia is a condition where bones are less dense than healthy bones, but it's not as bad as osteoporosis. Women with osteopenia might need treatment to reduce their risk of osteoporosis.

Our bones are living tissue that constantly break down and rebuild. This process keeps our bones strong and healthy. Menopause increases the risk of osteoporosis because